



PROCEDURE

Title/Subject: Productivity Standards			
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Productivity standards are in place for all therapists at The Florida Center. Therapists include mental health therapists/consultants, occupational therapists, and speech/language pathologists.

Productivity standards are in place to ensure the following:

- To increase access to specialized therapeutic care for children and families,
- To ensure full-time therapists are dedicating an adequate percentage of their work week to the provision of services for which they were hired,
- To ensure a steady flow of revenue to programs that rely on Medicaid/third-party reimbursement to stay afloat.

I. Determining Productivity

There are different categories when determining productivity:

- Billable Session-** activities include individual therapy, groups, evaluations, treatment plans, assessments, etc. (typically billed under Medicaid/Third Party Insurance, but could have another funding source)
- Other Services** – activities include case management, consultations, and mentoring (this category is determined by program). All activities under this category have other funding sources.
- Administrative Time.** Administrative time includes:
 - a) Additional time needed to complete paperwork,
 - b) Staff meetings,
 - c) In-services/staff trainings,
 - d) Supervision,
 - e) Special assignments/exceptions as approved by Supervisor,
 - f) Travel time (when providing out-of-office services),

Note: Paid holidays, required time off for winter break and PTO are not included in hours when determining productivity.

II. Productivity Standards/Expectations

1. There is a minimum average weekly productivity expectation for all therapists. See appendix for the breakdown for your specific role/program.
2. Therapists are allowed 20-25% (not to exceed 2 hours per day) of dedicated time each week to complete administrative duties and responsibilities.
3. Therapists should strive to maintain a cancellation rate of 20% or less. Therapists are expected to use cancellations as an opportunity to catch up on paperwork, such as, documentation, report writing and returning emails/calls, or to fill that time with consultations for other clients. Cancellations are defined as a client appointment that was scheduled that did not occur.
4. Therapists are required to enter Billable Sessions and Other Services (example: ITXXCC and CCM) into Lauris (the agencies electronic health records software program) via the "scheduler" section of Lauris. All notes attached to scheduled appointments must also be completed via the "scheduler" (see Lauris training manual for more details).
5. While progress notes should be completed the day of service, we recognize that this is not always possible. However, notes are required to be entered into Lauris no later than **2 business days** after the session occurred.
6. Evaluations/Assessments are required to be entered/completed in Lauris within **5 business days** of the occurrence.

Note: Time frames listed above to complete forms are REQUIRED, per Medicaid Guidelines.

III. Staff and Supervisor Responsibilities

1. Supervisors are required to work with the intake department to ensure there are adequate clients being referred to the program.
2. Supervisors are required to assign cases to therapists on a regular basis.
3. Therapists are expected to timely "close" clients they are no longer seeing.
4. Therapists are required to notify their supervisor if they are in need of additional cases.
5. Therapists may be asked to see clients in other programs and/or do perform other duties as assigned during slower productivity time periods (for example: A school-based therapist who is slow during the summer, may be asked to see clients on the waiting list for outpatient mental health, offer groups work, or conduct trainings/presentations).

IV. Remediation

Therapists struggling to meet any standards or expectations should discuss options with their immediate supervisor and devise a plan. If low productivity is out of the control of therapists (for example: lack of referrals) therapists will not be placed on a performance improvement plan. Therapists are expected to be proactive and communicate their need for additional cases to supervisors.

Therapists' productivity is monitored and reviewed quarterly to ensure adherence to standards. Therapists consistently not meeting productivity standards/expectations each quarter will be placed on a performance improvement plan. They will be given one quarter to improve their productivity percentage. The performance improvement plan will be terminated once the therapist has been able to meet the quarterly productivity standards/expectations.

Therapists who are unable to meet the productivity standards/expectations are at risk of disciplinary action that may include termination of employment.

I have read The Florida Center's Productivity Standards Procedure and I understand the procedure as it is written. I have been given a copy of the procedure along with the specific productivity expectations for my program/role.

Signature:

Date:

____/____/____

Appendix A

Weekly Breakdown: Outpatient Mental Health, Early Childhood Court, Fetal Alcohol Spectrum Disorders Clinic, Early Childhood Mental Health Consultation, School-Based Mental Health, and Family Support Team

With the support of leadership, each staff member is required to manage their 40-hour work week in a way that allows them to accomplish 100% of their duties within 40 hours. The *minimum* number of direct-client hours any one full-time staff member should have available to schedule per week is 30. The weekly productivity is as follows:

The work week is broken down as follows:

- a) **75% - 30 hours per week = 6 hours per day of direct client work.** This includes billable sessions + consultation (ITXXCC) and clinical case management (CCM). Direct client work is the bulk of what we do. It's working with the children, families, schools, etc., and includes all activities that have a funding source attached (insurance grants, etc.).
- b) **25% - 10 hours per week = 2 hours per day of administrative time.** This includes documentation time, meetings, supervision, travel, etc. This is the smaller part of our job which is documenting, report writing, attending meetings, trainings, etc. This is the part of our work that is not paid for by insurance companies.

Appendix B

Weekly Breakdown: Developmental Therapies

With the support of leadership, each therapist is required to manage their 40-hour work week in a way that allows them to accomplish 100% of their duties within 40 hours. The minimum number of therapy hours any one full-time developmental therapist should have available to schedule per week is 30. The weekly productivity for developmental therapies is as follows:

The work week is broken down as follows:

- a) 60% (24 hours) Billable sessions (a minimum of 50% (20hours))
- b) 20% (8 hours) Other services (acceptable monthly range 10%-30% (4-12 hours))
- c) 20% (8 hours) Administrative time

The combined total of both (a) and (b) should equal 80%.